

1. Administrative & Study Identification

Full Study Title: A Study of AZD0486 Plus Rituximab in Previously Untreated Follicular Lymphoma Patients **Short Title/Acronym:** SOUNDTRACK-F1 **Protocol Number:** D7401C00001 **NCT Number:** NCT06549595 **Posting ID:** 415042883095656607 **Sponsor/Company Name:** AstraZeneca **Investigational Product:** AZD0486 (Surovatamig) and Rituximab **Phase of Development:** PHASE3 **Trial Status:** RECRUITING (Currently recruiting participants) **Disease Areas:** Untreated Follicular Lymphoma **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To demonstrate the superiority of AZD0486 plus rituximab versus standard chemoimmunotherapy (Investigator's choice) in patients with previously untreated follicular lymphoma (FL). **Secondary Objectives:** To evaluate Overall Response Rate (ORR), Complete Response (CR) rate, and overall safety.

2.2 Background & Rationale Follicular lymphoma is the most common indolent lymphoma; advanced-stage disease is generally considered incurable despite recent advances. The current standard initial treatment for FL patients with high tumor burden requires chemoimmunotherapy combined with a CD20 monoclonal antibody (like rituximab), which induces high response rates but is associated with notable chemotherapy-related toxicities. AZD0486 is a novel CD19xCD3 fully human IgG4 bispecific T-cell engager. This study evaluates whether an investigational, chemotherapy-free approach combining AZD0486 with rituximab offers superior efficacy and better tolerability.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator **Blinding:** Open-label **Randomization:** Randomised, Multicentre

3.2 Study Sequence & Arms The study consists of 2 sequential parts: 1. **Safety Run-in:** This part will compare dose levels of AZD0486 in combination with rituximab in order to establish the Recommended Phase 3 Dose (RP3D). 2. **Phase III:** The Phase III part will assess the superiority of AZD0486 at RP3D in combination with rituximab, compared to Investigator's choice between 3 standard chemoimmunotherapy regimens. Phase 3 consists of 3 arms:

* **Arm A:** Treatment with AZD0486 plus rituximab Schedule A * **Arm B:** Treatment with AZD0486 plus rituximab Schedule B * **Arm C (Comparator arm):** One of the following standard regimens per Investigator's choice: R-CVP + rituximab maintenance, R-CHOP + rituximab maintenance, and B-R + rituximab maintenance.

3.3 Planned Enrollment & Duration Targeted Enrollment: 1,015 participants **Number of Sites:** Global, multicentre **Estimated Study Start:** 07-Aug-2024 **Estimated Primary Completion:** 26-Nov-2031

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant must be at least 18 years of age, inclusive, at the time of signing the ICF. 2. Histologically confirmed diagnosis of classic FL per WHO 2022 classification. 3. ECOG performance status of 0 to 2. 4. No prior systemic lymphoma-directed therapy. 5. Need for systemic treatment meeting at least 1 GELF criteria. 6. FDG-avid and measurable disease. 7. Stage II to IV and FLIPI 2-5 [Phase III only]. 8. Adequate liver, hematological, renal and cardiac function.

4.2 Exclusion Criteria 1. Follicular large B-cell lymphoma (WHO 2022 classification), formerly Follicular lymphoma Grade 3B (WHO 2016 classification) or suspicion for histologic transformation to high-grade/aggressive lymphoma. 2. Contra-indication to BR, RCVP, and R-CHOP. 3. Participants with or history of CNS lymphoma. 4. Presence of >5000 circulating lymphoma cells. 5. Active or uncontrolled infection requiring systemic therapy and which places participant at unacceptable risk if he/she were to participate in the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: AZD0486 combined with rituximab (Schedule A or Schedule B as defined by study arm). **Route of Administration:** Intravenous / Subcutaneous. **Comparator:** Up to 6 cycles of standard chemoimmunotherapy followed by rituximab maintenance.

5.2 Concomitant & Prohibited Therapy Permitted: Standard anti-infective prophylaxis and supportive care medications. **Prohibited:** Concurrent administration of other systemic lymphoma-directed therapies, live virus vaccinations, or other investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent,

Medical History, Baseline PET/CT, Labs | | **Baseline** | Day 0 | Randomization, First Dose, Safety Monitoring | | **Interim** | Every Cycle | Efficacy Assessment (CT/PET), Safety Labs (CRS/ICANS monitoring) | | **End of Study** | Up to 10 Years | Long-term Survival Follow-Up, Disease Progression Assessment |

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes * **SRI Primary 1:** Incidence, nature and severity of AEs and SAEs. Incidence and nature of study drug discontinuations, dose reductions, and dose delays due to AEs: Frequency, severity, and relationship to study drug of AEs and SAEs; dose modifications; changes in physical examination and safety procedures. * **SRI Primary 2:** Determination of the recommended Phase III dose (RP3D): The RP3D will be the dose of AZD0486 selected for the Phase 3 part based on safety data compiled during the safety run-in part. * **Phase 3 Dual Primary:** To demonstrate the superiority of AZD0486 plus rituximab compared to Investigator's choice of SoC chemoimmunotherapy: Progression-Free Survival (PFS), based on Lugano 2014 Response Criteria, as assessed by Blinded Independent Central Review (BICR).

7.2 Secondary Outcomes * **Safety Run-in and Phase 3:** Overall Response Rate (ORR) at End of Induction (EoI) (Investigator assessed). * **Safety Run-in and Phase 3:** Complete Response (CR) Rate. * **Safety Run-in and Phase 3:** Complete Response (CR) at EoI.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Extensive monitoring for bispecific antibody-specific toxicities, primarily Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline to the sponsor. **Data Monitoring Committee:** Independent committee to evaluate safety run-in data to establish the RP3D and monitor ongoing Phase III trial safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy outcomes; Safety Set for all participants receiving ≥ 1 dose of study drug. **Sample Size Justification:** 1,015 patients powered to detect a statistically significant improvement in primary endpoints. **Handling of Missing Data:** Standard censoring rules per regulatory guidance for time-to-event endpoints (Kaplan-Meier survival estimates).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits by the sponsor to ensure protocol compliance and patient safety.

11. Study Identifiers & Governance

Primary ID: D7401C00001 **Registry Number:** NCT06549595 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** SOUNDTRACK-F1 **Official Title:** A Phase III, Multicentre, Randomised, Open-label Study to Compare the Efficacy and Safety of AZD0486 Plus Rituximab Versus Chemotherapy Plus Rituximab in Previously Untreated Participants With Follicular Lymphoma (SOUNDTRACK-F1) **Summary Details:** This is a global, randomised, Phase III, multicentre, open-label study evaluating the efficacy, safety and the degree of added benefit of the AZD0486 plus rituximab combination compared to Investigator's choice of 3 standard immunochemotherapy regimen, conducted in participants with untreated FL.

12. Clinical Framework (Follicular Lymphoma Specific)

Primary Condition: Follicular Lymphoma (FL) **Diagnosis Threshold:** Histologically confirmed classic FL, Stage II-IV, FLIPI 2-5, meeting ≥ 1 GELF criteria **Medical Methodology:** T-cell engaging bispecific antibody therapy + CD20 targeted therapy **Optimization Target:** Progression-free survival and complete disease response

13. System & Ontology Metadata

To ensure standardized data capture, the following coding schema and semantic mappings are maintained in the clinical trial registry configuration:

Disease & Condition Classifications: * **Name / Preferred Name:** Lymphoma * **Preferred UMLS Name:** Newly diagnosed follicular lymphoma * **Semantic Type:** Neoplastic Process * **Definition:** A cancer originating in lymphocytes and presenting as a solid tumor of lymphoid cells. [<https://orcid.org/0000-0002-0736-9199>] * **MeSH Code:** D008223 * **HPO Code:** HP:0002665 * **SNOMED ID:** 118600007 * **SNOMED Hierarchy Mapping:** Lymphoreticular tumor (277606000), Lymphoproliferative disorder (277466009), Disorder of hematopoietic cell proliferation (414026006), Disease (64572001), Neoplasm of hematopoietic cell type (414825006), Neoplastic disease (55342001), Malignant neoplasm of lymphoid, hematopoietic and/or related tissue (269475001), Malignant neoplastic disease (363346000)

Concomitant/System Mapped Drug Entity (Registry Linking): * **Drug Name / Preferred Name:** agalsidase beta * **Trade Name(s):** Fabrazyme
* **ATC Code:** A16AB04 * **Drug Semantic Type:** [None] * **EMA URL:**
https://www.ema.europa.eu/en/documents/product-information/fabrazyme-epar-product-information_en.pdf

Criteria Mapping: * **Criteria Type:** PHASE_REQUIREMENT * **Criteria Text:** Trial must be in PHASE3 * **Phase ID:** PHASE3

14. Observation & Follow-Up Schedule

Total Duration: Up to 10 years (120 months) **Onsite Visit Cadence:** Day 1 of each cycle during the active treatment phase, transitioning to extended surveillance. **Remote Monitoring Frequency:** As required for long-term survival follow-up.

15. Sign-off & Approval

Role	Name	Signature	Date	:	---	:	---	:	---	:	---
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---